

Pregnancy + symptomatic SVT

TREAT: Vagal maneuvers first; **adenosine** commonly used for acute regular SVT

CONTRAST: Hemodynamic instability -> **synchronized cardioversion**

Pulmonology and Critical Care

PFT Pattern Boundaries

FEV1/FVC <70% with scooped-out flow-volume loop

ANSWER: Obstructive physiology

CONTRAST: Restriction has **low TLC** with **preserved or high FEV1/FVC**

NEXT: Use **bronchodilator response** and **DLCO** to separate asthma, COPD, and emphysema; LLN can refine clinical interpretation

Obstruction improves by >12% and >200 mL FEV1 after bronchodilator

Dx: Asthma

CONTRAST: Persistent post-bronchodilator obstruction in a smoker -> **COPD**

RULE: Reversibility supports asthma, but **fixed obstruction** can occur in long-standing asthma

Restrictive spirometry and low TLC

ANSWER: True restriction

RULE: TLC confirms restriction; low FVC alone can be **pseudo-restriction** from **air trapping**

CONTRAST: Obstruction lowers FEV1/FVC and often has normal or high TLC

Restriction + low DLCO

ANSWER: Intrinsic parenchymal lung disease such as **ILD** or **sarcoidosis**

CONTRAST: Restriction with **normal DLCO** -> obesity, neuromuscular weakness, or chest wall disease

Obstructive PFT + low DLCO

Dx: Low DLCO in obstructive disease favors **emphysema-predominant COPD**

CONTRAST: Asthma classically has normal or elevated DLCO; chronic bronchitis often has normal DLCO

TRAP: Low DLCO in presumed asthma should prompt another diagnosis such as COPD overlap, ILD, or remodeling

Asthma versus COPD on PFTs — board comparison

ASTHMA: Episodic obstruction; often large bronchodilator response; **normal or high DLCO**; may fully reverse between attacks

COPD: **Persistent** post-bronchodilator FEV1/FVC <0.70; incomplete reversibility; emphysema -> **low DLCO** + hyperinflation (↑ TLC/RV)

CONTRAST: ACO (asthma-COPD overlap) can show mixed features — treat for both phenotypes when both are present